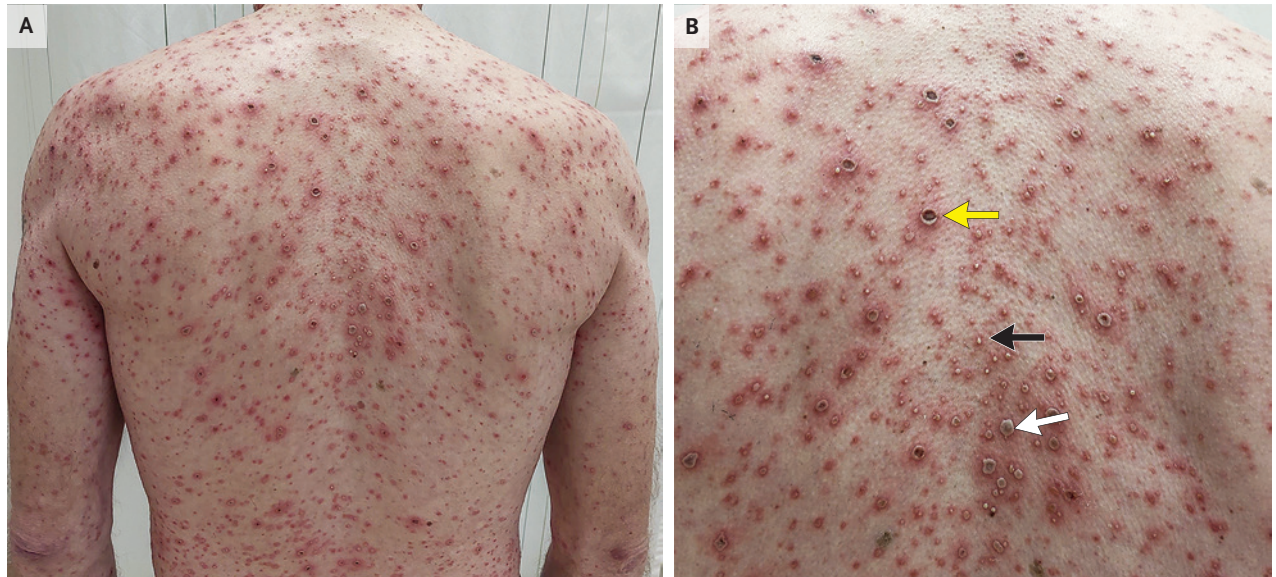


IMAGES IN CLINICAL MEDICINE

Primary Varicella Infection



A PREVIOUSLY HEALTHY 44-YEAR-OLD MAN PRESENTED TO THE DERMATOLOGY clinic with a 4-day history of an itchy rash and a 2-day history of fever and malaise. The rash had first appeared on the scalp and then spread across the body within 24 hours. The patient reported no history of chicken pox or varicella vaccination. Physical examination was notable for widespread erythematous papules and pustules on the back (Panel A), chest, arms, legs, face, and scalp. The eruption was pleomorphic, with lesions of different sizes and stages of evolution (Panel B), including umbilicated lesions with central necrosis (yellow arrow), pustules (black arrow), and vesicles (white arrow). Laboratory testing was notable for mild elevations in aminotransferase levels. A polymerase-chain-reaction assay of fluid from a vesicle was positive for varicella-zoster virus. A diagnosis of primary varicella infection — or chicken pox — was made. Primary varicella infection in adults is often more severe than in children, which highlights the importance of childhood vaccination. Adults with varicella infection — even immunocompetent ones, such as this patient — have an increased risk of complications, such as pneumonia and encephalitis. Treatment with oral valacyclovir was administered. The illness abated without complications, and only residual hyperpigmentation from the rash remained at the 4-week follow-up.

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